Chronic pain initial evaluation, low back pain down the leg

A full doctor-patient conversation and the finished note it should produce, laid out in the Sully Chronic Pain Initial Evaluation template. Use it to test an AI scribe, train new scribes, or see what a complete note captures.

Patient	46-year-old woman (fictional)
Visit	New patient evaluation, in person
Length	About 16 minutes, 60 turns
Matching template	Chronic Pain Initial Evaluation · sully.ai/templates/chronic-pain-initial-evaluation

A Visit transcript

Doctor	Ms. Brennan? Hi, I'm Dr. Shah. Thanks for filling out all those forms, I know there were a lot.
Patient	There were so many. Hi.
Doctor	I read through them, but I'd rather hear it from you. What's bringing you in?
Patient	My back, and my left leg. It's been, God, almost a year now. I was moving my daughter into her dorm last August, I picked up one of those plastic bins, and I felt something, like a pop, low on the left side. And it never really went away.
Doctor	And the leg, when did that start?
Patient	Couple of weeks after. It goes down the back of my butt, the back of the thigh, and the outside of my calf. Sometimes into the top of my foot.
Doctor	Into the top of the foot, toward the big toe?
Patient	Yeah, kind of between the big toe and the next one.
Doctor	And what's worse right now, the back or the leg?
Patient	The leg. Definitely. The back is like a four. The leg some days is an eight.
Doctor	What does the leg pain feel like?
Patient	Burning. And electric, like a zap, when I bend over. And there's a numb patch on the side of my calf.
Doctor	What makes it worse?
Patient	Sitting. Like, driving more than twenty minutes is awful. Bending. Coughing, actually, if I sneeze it shoots down.

Doctor	And better?
Patient	Lying on my back with my knees up. Walking a little actually helps, as long as it's short.
Doctor	Any weakness in the leg? Tripping, foot slapping, trouble going up on your toes or your heels?
Patient	I don't think so. Maybe I catch my toe sometimes? I'm not sure if that's new.
Doctor	OK, I'll check that. I have to ask a couple of important questions. Any trouble controlling your bladder or bowels? Numbness in the groin or the area you'd sit on a saddle?
Patient	No. Nothing like that.
Doctor	Fevers, night sweats, weight loss you can't explain, history of cancer?
Patient	No.
Doctor	Good. What have you tried so far?
Patient	I did physical therapy, twelve visits I think, in the spring. It helped the back a little. Not the leg. I did ibuprofen, which upsets my stomach. My primary put me on gabapentin, 300 at night, and it makes me really foggy in the morning. And I did a Medrol dose pack, which was great for like five days.
Doctor	Any injections?
Patient	No. That's kind of why I'm here.
Doctor	Have you been on any opioid medication for this? Tramadol, hydrocodone?
Patient	I had some Norco after my C-section years ago. Not for this. I don't want to go that route, honestly. My brother had a problem with pills.
Doctor	I appreciate you telling me, and I agree, it's not where we'd start. How is this affecting your life? Work, sleep, mood.
Patient	I'm a dental hygienist, so I'm leaning over people all day. I've gone from four days to three. Sleep is bad, I wake up when I roll over. Mood... *(pause)* I've been more down. I'm short with my kids. I used to run, and I can't.
Doctor	That's a lot to be carrying for a year. Have you had thoughts that you'd be better off dead, or of hurting yourself?
Patient	No. No, nothing like that. Just frustrated.
Doctor	OK. Thank you. If that ever changes, I want you to tell us. Any other medical problems?
Patient	Hypothyroid. I take levothyroxine. That's it.
Doctor	Smoking, alcohol, anything else?
Patient	No smoking. Glass of wine maybe twice a week.
Doctor	And the MRI, I see you had one in July. Let me pull it up. *(pause)* So, at L4-5 there's a disc bulge pushing to the left, and it's pressing on the L5 nerve root as it comes out. That matches where your leg pain goes really well.
Patient	Oh. OK. Nobody actually explained that to me.
Doctor	Let me examine you and then we'll talk plan. *(exam)* Bend forward as far as is comfortable. OK. Lean back. Does that hurt?

Patient	Not as much as forward.
Doctor	Lie back for me. I'm going to lift your left leg straight. Tell me when you feel it.
Patient	There. Ow. That's it, that's the zap.
Doctor	About 40 degrees. And the right?
Patient	Just tight. No zap.
Doctor	Push up against my hand with your big toe. Now the right. *(pause)* Left is a little weaker. Walk on your heels for me. Now your toes. OK. Feel this on the side of your calf? Compare to the right.
Patient	It's duller on the left.
Doctor	Reflexes are symmetric. OK, you can sit up.
Doctor	So, here's what I think. This is a left L5 radiculopathy, a pinched L5 nerve from that disc. The leg pain, the numb patch, the positive leg raise and the mild weakness lifting your big toe all line up with the MRI. The good news is that most of these improve without surgery. The mild weakness is something I want to watch.
Patient	OK.
Doctor	Since PT and medication haven't been enough, I'd recommend an epidural steroid injection. For your pattern, a transforaminal injection at left L4-5, done under X-ray. It puts anti-inflammatory medicine right where the nerve is irritated. Many people get good relief, often enough to get back into therapy and strengthen.
Patient	Does it hurt?
Doctor	It's mostly pressure. We numb the skin. It takes about fifteen minutes. Risks are low but real: a temporary flare in pain, infection, bleeding, and rare nerve injury. Steroids can bump your blood sugar for a few days. I'll give you the written consent to read.
Patient	OK. I'd like to try it.
Doctor	For the gabapentin fogginess, let's try something different. Instead of 300 at night all at once, we can switch to duloxetine. It helps nerve pain, and it can help mood too. 30 milligrams in the morning for a week, and if you tolerate it, we go to 60.
Patient	Is that an antidepressant?
Doctor	It is, but here we're using it for nerve pain. It's one of the better-studied options for that. Taper the gabapentin over a week as you start it. And I'd like you back in PT after the injection, with a focus on core and nerve-glide work.
Patient	OK.
Doctor	Call us right away if you lose bladder or bowel control, get numb in the groin, or the foot gets clearly weaker, like you can't lift it. That's same-day.
Patient	Got it. Thank you. Really, this is the first time it's made sense.
Doctor	I'm glad. We'll get you scheduled.

B Finished note

Patient: Colleen Brennan (fictional), 46 F. Referred by PCP. New patient.

Chief complaint: Left low back and left leg pain, ~12 months.

History of present illness: Onset ~12 months ago after lifting a storage bin, felt a pop in the left low back. Left leg pain began ~2 weeks later, radiating to the posterior buttock, posterior thigh, lateral calf and dorsum of foot (first web space). Leg pain worse than back pain. Pain scores: back 4/10, leg up to 8/10. Quality: burning, electric with bending. Numbness lateral left calf. Worse with sitting (>20 min driving), bending, coughing, sneezing. Better supine with knees flexed and with short walks. Possible occasional toe catching.

Red flags: Denies bowel or bladder dysfunction, saddle anesthesia, fever, night sweats, unexplained weight loss, history of malignancy.

Prior treatment: PT ×12 visits (spring), partial back relief, no leg relief. Ibuprofen, GI upset. Gabapentin 300 mg qhs, morning sedation. Methylprednisolone dose pack, ~5 days of relief. No interventional procedures.

Opioid history and risk: No opioid therapy for this condition. Remote post-cesarean hydrocodone. Family history of opioid use disorder (brother). Patient declines opioids.

Function and mood: Dental hygienist, reduced from 4 to 3 days/week. Sleep disrupted. Low mood, irritability, stopped running. Denies suicidal ideation.

PMH: Hypothyroidism. **Medications:** Levothyroxine, gabapentin 300 mg qhs. **Social:** Non-smoker. Alcohol ~2 glasses wine/week.

Imaging: MRI lumbar spine (recent), reviewed and interpreted: L4-5 left paracentral disc protrusion with left L5 nerve root impingement.

Examination: Lumbar flexion reproduces leg pain; extension better tolerated. Straight leg raise positive on the left at ~40°, negative on the right. EHL strength 4+/5 left, 5/5 right. Heel and toe walk intact. Decreased light touch lateral left calf. Reflexes symmetric.

Assessment: Left L5 lumbar radiculopathy secondary to L4-5 disc protrusion, concordant with MRI. Mild EHL weakness. Chronic pain with functional and mood impact. Failed conservative care (PT, NSAID, gabapentinoid, oral steroid).

Plan:

1. Left L4-5 transforaminal epidural steroid injection under fluoroscopy. Risks, benefits and alternatives discussed; written consent provided.
2. Start duloxetine 30 mg qAM ×1 week, then 60 mg if tolerated. Taper gabapentin over 1 week.
3. Resume PT after injection: core stabilization and neural mobilization.
4. No opioid therapy.
5. Return precautions: bowel or bladder dysfunction, saddle anesthesia, progressive foot weakness, call same day.
6. Follow-up 2 to 3 weeks after injection; re-check EHL strength.

How to use this sample. Run the transcript through your scribe and compare its draft with the finished note. Check the details that are easy to miss: doses, which side of the body, what the patient said no to, and every follow-up step. For 536 more transcripts, many with audio and reference notes, see github.com/aniquethaplawala/open-clinical-encounters.

Rather not write the note yourself? Sully's AI Scribe listens to the visit and drafts the note in your template, ready for you to review and sign.

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